

HOUSE No. 1117**The Commonwealth of Massachusetts**

PRESENTED BY:

Manny Cruz and Lindsay N. Sabadosa*To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:*

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act promoting and enhancing the sustainability of birth centers and the midwifery workforce.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Manny Cruz</i>	<i>7th Essex</i>	<i>1/16/2025</i>
<i>Natalie M. Higgins</i>	<i>4th Worcester</i>	<i>2/3/2025</i>
<i>Susannah M. Whipps</i>	<i>2nd Franklin</i>	<i>2/12/2025</i>
<i>David Paul Linsky</i>	<i>5th Middlesex</i>	<i>2/13/2025</i>
<i>Bruce E. Tarr</i>	<i>First Essex and Middlesex</i>	<i>2/26/2025</i>
<i>Christopher J. Worrell</i>	<i>5th Suffolk</i>	<i>3/6/2025</i>
<i>Kevin G. Honan</i>	<i>17th Suffolk</i>	<i>3/7/2025</i>
<i>James C. Arena-DeRosa</i>	<i>8th Middlesex</i>	<i>3/13/2025</i>
<i>Rob Consalvo</i>	<i>14th Suffolk</i>	<i>3/14/2025</i>
<i>Rodney M. Elliott</i>	<i>16th Middlesex</i>	<i>3/14/2025</i>
<i>Margaret R. Scarsdale</i>	<i>1st Middlesex</i>	<i>3/17/2025</i>
<i>Samantha Montaño</i>	<i>15th Suffolk</i>	<i>3/17/2025</i>
<i>Sean Reid</i>	<i>11th Essex</i>	<i>3/17/2025</i>
<i>Daniel Cahill</i>	<i>10th Essex</i>	<i>3/18/2025</i>
<i>James B. Eldridge</i>	<i>Middlesex and Worcester</i>	<i>3/18/2025</i>
<i>Erika Uyterhoeven</i>	<i>27th Middlesex</i>	<i>3/20/2025</i>
<i>Joan B. Lovely</i>	<i>Second Essex</i>	<i>3/24/2025</i>
<i>Steven Owens</i>	<i>29th Middlesex</i>	<i>3/25/2025</i>

<i>Amy Mah Sangiolo</i>	<i>11th Middlesex</i>	<i>3/25/2025</i>
<i>Michael D. Brady</i>	<i>Second Plymouth and Norfolk</i>	<i>3/27/2025</i>
<i>Jeffrey Rosario Turco</i>	<i>19th Suffolk</i>	<i>3/31/2025</i>
<i>Michelle L. Badger</i>	<i>1st Plymouth</i>	<i>4/1/2025</i>
<i>Mike Connolly</i>	<i>26th Middlesex</i>	<i>4/4/2025</i>
<i>Vanna Howard</i>	<i>17th Middlesex</i>	<i>4/4/2025</i>
<i>Rebecca L. Rausch</i>	<i>Norfolk, Worcester and Middlesex</i>	<i>4/4/2025</i>
<i>Tommy Vitolo</i>	<i>15th Norfolk</i>	<i>4/7/2025</i>
<i>John J. Marsi</i>	<i>6th Worcester</i>	<i>4/18/2025</i>
<i>Michelle M. DuBois</i>	<i>10th Plymouth</i>	<i>5/27/2025</i>
<i>Marjorie C. Decker</i>	<i>25th Middlesex</i>	<i>6/4/2025</i>
<i>Tackey Chan</i>	<i>2nd Norfolk</i>	<i>7/2/2025</i>
<i>Tram T. Nguyen</i>	<i>18th Essex</i>	<i>10/16/2025</i>
<i>Brandy Fluker-Reid</i>	<i>12th Suffolk</i>	<i>10/17/2025</i>
<i>Natalie M. Blais</i>	<i>1st Franklin</i>	<i>10/28/2025</i>
<i>Patrick M. O'Connor</i>	<i>First Plymouth and Norfolk</i>	<i>11/4/2025</i>
<i>James K. Hawkins</i>	<i>2nd Bristol</i>	<i>11/21/2025</i>
<i>Jack Patrick Lewis</i>	<i>7th Middlesex</i>	<i>11/21/2025</i>
<i>David M. Rogers</i>	<i>24th Middlesex</i>	<i>1/6/2026</i>

HOUSE No. 1117

By Representatives Cruz of Salem and Sabadosa of Northampton, a petition (accompanied by bill, House, No. 1117) of Manny Cruz, Natalie M. Higgins and others relative to coverage of birth centers and the midwifery workforce. Financial Services.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act promoting and enhancing the sustainability of birth centers and the midwifery workforce.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 10 of the General Laws are hereby amended by inserting after
2 section 35TTT the following section:-

3 Section 35UUU. (a) There shall be established and set up on the books of the
4 commonwealth a separate fund known as the Midwifery Workforce Development Fund. The
5 fund shall be administered by the secretary of the executive office of health and human services
6 in consultation with the advisory committee established pursuant to subsection (d); provided, that
7 the secretary may delegate administration of part or all of the fund to an organization
8 representing certified professional midwives. There shall be credited to the fund: (i) revenue
9 from appropriations or other money authorized by the general court and specifically designated
10 to be credited to the fund; (ii) federal grants and other federal funds designated for maternal
11 health; (iii) interest earned on such revenues; and (iv) all other funds from public and private
12 sources credited or transferred into the fund, including gifts, public or private grants, settlements

and donations. Any unexpended balance in the fund at the end of a fiscal year shall not revert to the General Fund and shall remain available for expenditure in subsequent fiscal years. No expenditure made from the fund shall cause the fund to become deficient at any point. Any fiscal year-end balance in the fund that is subject to appropriation shall not be subject to section 5C of chapter 29. The fund shall not be subject to appropriation.

(b) Expenditures from the fund shall be used for:

(i) providing assistance to students in education and training programs to qualify them to become licensed certified professional midwives, including, but not limited to:

(A) program costs, tuition, books and fees related to the cost of education and training for participants in training and educational programs, including distance learning programs and including tuition and other costs related to obtaining a midwifery degree or Midwifery Bridge Certificate;

(B) costs related to obtaining an applicable license, including, but not limited to, examination and licensing fees;

(C) stipends for program participants; and

(D) stipends for childcare and transportation for program participants;

(ii) a student loan forgiveness program for certified professional midwives and certified nurse midwives; provided, that the program may require participants to practice in an underserved area or serve marginalized populations, as defined by the secretary in consultation with the advisory committee;

(iii) a grant program to fund local initiatives supporting the development of educational programs for certified professional midwives;

(iv) retention of practicing midwives, and mental health and psychosocial support for certified professional midwives and certified nurse midwives; and

(v) with the approval of the advisory committee, other purposes consistent with the goals of the fund.

(c) Annually, not later than December 1, the secretary of health and human services shall report on the activities of the fund to the clerks of the house of representatives and the senate and to the house and senate committees on ways and means and the joint committee on children, families and persons with disabilities. The report shall include: (i) an accounting of expenditures made from the fund with a description of the authorized purpose of each expenditure; (ii) an accounting of amounts credited to the fund; and (iii) any unexpended balance remaining in the fund. The report may also include analysis and recommendations prepared by the advisory committee, which may include recommendations for changes to this section. The report shall be published on the public web site of the executive office of health and human services or the department of public health.

(d) There shall be an advisory committee to make recommendations concerning the operation and expenditures of the fund. The advisory committee shall consist of the following members: a representative of the Massachusetts chapter of the National Association of Certified Professional Midwives, who shall serve as chair, a representative of the American College of Nurse Midwives; a member of the Pregnancy, Infancy and Early Childhood Division of the Department of Public Health; a representative of the Massachusetts League of Community

Health Centers; and the following members chosen by the chair: a representative of a midwifery educational program; a representative of a community college; a representative of an organization concerned about reproductive justice and health equity; a representative of a safety-net hospital; and a representative of a free-standing birth center.

SECTION 2. Chapter 32A of the General Laws is hereby amended by inserting after section 17W the following sections:-

Section 17X. (a) The commission shall ensure that the rate of payment for services provided to an active or retired employee of the commonwealth who is insured under the group insurance commission that are rendered by a certified nurse-midwife authorized to engage in the practice of nurse-midwifery by the board of registration in nursing pursuant to section 80C of chapter 112 that is within the scope of the certified nurse-midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

(b) The commission shall ensure that the rate of payment for services for services provided to an active or retired employee of the commonwealth who is insured under the group insurance that are rendered by a licensed certified professional midwife authorized to engage in the practice of midwifery by the board of registration in midwifery pursuant to section 110 of chapter 13 that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

Section 17Y. The commission shall ensure that the rate of payment for services provided to an active or retired employee of the commonwealth who is insured under the group insurance

commission that are rendered by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a hospital, regardless of the site of services, or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the commission; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 3. The third paragraph of section 10A of chapter 118E of the General Laws, as inserted by section 40 of chapter 186 of the acts of 2024, is hereby amended by adding the following sentences:- The payment rate for a service provided by a licensed certified professional midwife that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services. The payment rate for services provided by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a hospital, regardless of site of services or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the division; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 4. Chapter 175 of the General Laws is hereby amended by inserting after section 47YY the following sections:-

Section 47ZZ. (a) A policy, contract, agreement, plan or certificate of insurance issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by a certified nurse-midwife authorized to engage in the practice of nurse-midwifery by the board of registration in nursing pursuant to section 80C of chapter 112 that is within the scope of the certified nurse-midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

(b) A policy, contract, agreement, plan or certificate of insurance issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by a licensed certified professional midwife authorized to engage in the practice of midwifery by the board of registration in midwifery pursuant to section 110 of chapter 13 that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

Section 47AAA. A policy, contract, agreement, plan or certificate of insurance issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a

hospital, regardless of the site of services, or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the payer; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 5. Chapter 176A of the General Laws is hereby amended by inserting after section 8ZZ the following sections:-

Section 8AAA. (a) A contract between a subscriber and the corporation under an individual or group hospital service plan that is delivered, issued or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by a certified nurse-midwife authorized to engage in the practice of nurse-midwifery by the board of registration in nursing pursuant to section 80C of chapter 112 that is within the scope of the certified nurse-midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services.

(b) A contract between a subscriber and the corporation under an individual or group hospital service plan that is delivered, issued or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by a licensed certified professional midwife authorized to engage in the practice of midwifery by the board of registration in midwifery pursuant to section 110 of chapter 13 that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

Section 8BBB. A contract between a subscriber and the corporation under an individual or group hospital service plan that is issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a hospital, regardless of the site of services, or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the payer; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 6. Chapter 176B of the General Laws is hereby amended by inserting after section 4ZZ the following sections:-

Section 4AAA. (a) Any subscription certificate under an individual or group medical service agreement that is delivered, issued or renewed within the commonwealth shall provide that the rate of payment for services rendered by a certified nurse-midwife authorized to engage in the practice of nurse-midwifery by the board of registration in nursing pursuant to section 80C of chapter 112 that is within the scope of the certified nurse-midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services.

(b) Any subscription certificate under an individual or group medical service agreement that is delivered, issued or renewed within the commonwealth shall provide that the rate of payment for services rendered by a licensed certified professional midwife authorized to engage in the practice of midwifery by the board of registration in midwifery pursuant to section 110 of chapter 13 that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

Section 4BBB. Any subscription certificate under an individual or group medical service agreement that is issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a hospital, regardless of the site of services, or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the payer; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 7. Chapter 176G of the General Laws is hereby amended by inserting after section 4RR the following sections:-

Section 4SS. (a) Any individual or group health maintenance contract that is issued or renewed within or without the commonwealth shall provide that the rate of payment for services

rendered by a certified nurse-midwife authorized to engage in the practice of nurse-midwifery by the board of registration in nursing pursuant to section 80C of chapter 112 that is within the scope of the certified nurse-midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

(b) Any subscription certificate under an individual or group medical service agreement that is delivered, issued or renewed within the commonwealth shall provide that the rate of payment for services rendered by a licensed certified professional midwife authorized to engage in the practice of midwifery by the board of registration in midwifery pursuant to section 110 of chapter 13 that is within the scope of the licensed certified professional midwife's authorization to practice shall be no less than the payment rate for the same service if performed by a physician, regardless of the site of services, or length of stay.

Section 4TT. Any individual or group health maintenance contract that is issued, delivered or renewed within or without the commonwealth shall provide that the rate of payment for services rendered by or within a licensed freestanding birth center regulated under section 51M of chapter 111, including newborn care and care provided prior to an intrapartum transfer to another facility, shall be no less than the payment rate for the same services if performed at a hospital, or length of stay. Such payment shall include a reasonable facility fee to be negotiated between the birth center and the payer; provided, that the facility fee shall be no less than a minimum fee amount determined by the department of public health in consultation with the center for health information and analysis, the health policy commission, and operators and prospective operators of birth centers in the commonwealth.

SECTION 8. (a) No later than 12 months after the effective date of this act, the department of public health shall establish licensing requirements, plan review checklists and facility guidelines specific for birth centers that reflect the particular needs of birth centers and low-risk childbearing people. The requirements and guidelines shall be developed in consultation with operators of birth centers in the commonwealth, people involved with the development of prospective birth centers in the commonwealth, and groups concerned with improving birth outcomes, midwifery and birth centers, including, but not limited to, the American College of Nurse-Midwives, the National Association of Certified Professional Midwives, the American Association of Birth Centers, and the Commission for the Accreditation of Birth Centers.

(b) No later than 90 days after the effective date of this act, the department of public health shall waive plan review fees for birth centers which demonstrate substantial financial hardship. The department shall issue guidelines and an application process for birth centers seeking a waiver of plan review fees under this subsection; provided, that the guidelines and application process shall be developed in consultation with operators of birth centers in the commonwealth, people involved with the development of prospective birth centers in the commonwealth, and groups concerned with improving birth outcomes, midwifery and birth centers, including, but not limited to, the American College of Nurse-Midwives and the National Association of Certified Professional Midwives.

SECTION 9. No later than 18 months after the effective date of this act, the department of public health, or a third party designated by the department, shall participate in the Community Birth Data Registry organized by the Foundation for Health Care Quality, or an equivalent validated national data collection registry, for the purposes of quality improvement

231 and research, and direct all midwives in the commonwealth to record data on community births
232 with the registry.

233 Aggregate data as defined by the Board of Registration in Midwifery and as collected by
234 the registry shall be made available to the Department of Public Health Vital Records Division,
235 and shall be included in reports on births and birth outcomes reported by the department.
236 Researchers who meet reasonable standards and who have passed an appropriate ethics review
237 established by the department may have access to data collected by the registry. Confidential
238 birth data involving identifiable individuals shall under no circumstances be used to determine
239 whether a person is eligible for a license, privilege, right, grant, or benefit, or whether such
240 should be revoked. No individual or entity may query the dataset for purposes of investigation or
241 discipline.

242 No later than 6 months after the effective date of this act, the department of public health
243 shall issue a report on progress towards establishing a community birth data registry in
244 Massachusetts. The report shall identify any barriers identified by the department, how the
245 department intends to overcome the barriers, and the cost to the Commonwealth and midwives
246 and birth centers of participation. The report shall be filed with the house and senate committees
247 on ways and means and the joint committees on public health and children, families and persons
248 with disabilities, and published on the web site of the department.

249 For the purposes of this section, the term “community births” shall mean a birth that is
250 planned to occur in a home, birth center, or other location other than a hospital, and including
251 those that involve or include transfer to a hospital.